

Guideline Title: Seizures

Guideline Number: GUID-3203-M015

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Revision dates: 4/30/24	Developed by: Air Care Team
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- I. **PURPOSE:** To recognize the patient with a seizure disorder and prevent active seizing. Assure adequate oxygenation, ventilation, tissue perfusion, and provide expeditious transport to the appropriate medical facility.
- II. **DEFINITIONS:**
- III. **DEPARTMENT GUIDELINE:**
 - a. Establish care as defined by *GUID3203-G002 – General Patient Care*.
 - b. Prepare a safe environment for the patient.
 - c. Perform a blood glucose check. If < 60 mg/dL treat per *GUID3203-M021 Hypoglycemia*.
 - d. If patient is actively seizing
 - i. **Midazolam 5 mg IV/IN or 10 mg IM.** Repeat as necessary q 2-5 minutes up to 20 mg. **-OR-**
 - ii. **Lorazepam 2 mg IV/IM** (if available). Repeat as necessary q 2-5 minutes up to 6 mg.
 - e. If multiple doses of benzodiazepines given and patient is still in status epilepticus, consider initiating ONE of the following anticonvulsant drugs
 - i. **Levetiracetam 60 mg/kg IV.** Maximum dose 4.5 grams over 5 minutes. *This is the preferred second line agent after benzodiazepines.* For doses >1000 mg, dilute vials in 100 ml of 0.9% NS and administer over 15 minutes.
 - ii. **Phenytoin 15-20 mg/kg IV infusion at 25-50 mg/min**
 - iii. **Fosphenytoin 15-20 mg PE/kg IV infusion at 100-150 mg PE/min**
 - f. If seizure activity continues, consider **Pentobarbital 10-20 mg/kg IV** infused over 30 minutes, followed by **Pentobarbital infusion at 1mg/kg/hr**
 - g. Consider rapid sequence intubation per *GUID3203-G008 Rapid Sequence Intubation*.
 - i. If intubation required, use short acting neuro-muscular blockers as long acting neuromuscular blockade will mask physical evidence of seizures.
 - ii. Consider placement of an orogastric or nasogastric tube per *GUID3203-PR010 Gastric Tube Insertion* to decrease risk of aspiration.
- IV. **DOCUMENTATION:** EMS Charts
- V. **REFERENCES:**